

Cross-Jurisdictional Medical Travel and Social Welfare Protections within the Common Travel Area: A Clinical and Statutory Appraisal

Executive Summary and Clinical Context

The following analysis provides an exhaustive, evidence-based synthesis of the legal, clinical, and logistical frameworks surrounding an individual holding dual Irish-British citizenship who resides primarily in the Republic of Ireland but requires frequent extraterritorial travel to access specialized healthcare. The central point of adjudication lies in the intersection between the residency and physical presence requirements stipulated by the Irish Department of Social Protection for the receipt of Disability Allowance, and the fundamental, inherent rights guaranteed to the claimant as a British citizen operating under the Common Travel Area (CTA) agreements.¹

The claimant presents with a heavily documented clinical history of Complex Post-Traumatic Stress Disorder (CPTSD), a condition rooted in severe, sustained trauma, including childhood adversity and adult domestic violence.³ The management of this complex psychiatric presentation necessitates highly specialized, trauma-informed clinical care, alongside adjunctive pharmacological interventions encompassing medicinal cannabis and advanced neuropeptide therapies.[3, 3] Due to the stringent, highly prohibitive regulatory environment enforced by the Health Products Regulatory Authority (HPRA) in Ireland, combined with a structural deficit in trauma-informed clinical pathways within the Irish Health Service Executive (HSE), the claimant is systematically compelled to seek these life-sustaining treatments within the United Kingdom.⁴ This report collates an extensive matrix of primary evidence, encompassing passport identification, detailed chronological travel itineraries, clinical correspondence, and procurement ledgers for medical devices and therapeutics. By triangulating these datasets with the statutory provisions of the Social Welfare Consolidation Act 2005 and the bilateral CTA Memorandum of Understanding (2019), the evidence demonstrates that the claimant's frequent absences from the Irish State are explicitly non-discretionary. They constitute essential medical travel protected by international reciprocal agreements, thus fulfilling the criteria for administrative exceptions to standard social welfare physical presence requirements.

Jurisdictional Identity and Common Travel Area Protections

The claimant's legal right to access healthcare across jurisdictions relies entirely upon a verified

status as a dual national. Documentary evidence confirms the claimant holds active citizenship in both the Republic of Ireland and the United Kingdom of Great Britain and Northern Ireland.³ The Irish passport (Document Number PP5175383, issued on 27 January 2025, expiring 26 January 2027) identifies the holder's nationality as "Éireannach/Irish" and the place of birth as "Gaillimh/Galway".³ The passport records the surname as "Mac Liatháin" and forename as "Cian," with an official observation stating: "BIRTH CERT NAME: CIAN PIERCE LYONS".³ Concurrently, the British passport (Document Number 155887673, issued on 12 May 2025, expiring 12 May 2035) explicitly lists the holder's nationality as "British Citizen," while also recording the place of birth as "Galway".³ The British passport features a formal administrative observation linking the two distinct identities, noting: "THE HOLDER HAS A IRELAND PASSPORT, NUMBER PP5175383, ISSUED ON 27 JANUARY 2025, IN THE ONAME CIAN MACLIATHAIN".³

This dual status carries profound legal and administrative significance. The claimant was born in the Republic of Ireland, affording them birthright Irish citizenship, yet concurrently holds full British citizenship.³ This activates a unique and highly protected tier of rights under the Common Travel Area. The CTA is a deeply entrenched arrangement that predates the European Union, facilitating the seamless movement of British and Irish citizens between the two jurisdictions.² Following the United Kingdom's withdrawal from the European Union, the British and Irish governments signed a Memorandum of Understanding (MoU) in May 2019 to reaffirm the CTA and guarantee its continuity in all circumstances.¹

Under the CTA, British citizens residing in Ireland, and Irish citizens residing in the UK, retain the absolute right to live, work, study, and access both social security and healthcare in either jurisdiction without requiring immigration permission.² The Withdrawal of the United Kingdom from the European Union (Consequential Provisions) Act 2019 further enshrines these reciprocal rights in Irish domestic law, ensuring equal access to healthcare and social welfare.² The primary insight derived from this legal framework is that the claimant's pursuit of private healthcare within the UK—including consultations at specialized psychiatric clinics in London and the utilization of dispensaries in Belfast—is a direct exercise of their inherent rights as a British citizen.^[3, 3] The fact that the claimant was born in Ireland and maintains a primary domestic base there for the purposes of claiming an Irish Disability Allowance does not extinguish or subordinate their status as a British citizen.³ Consequently, any domestic administrative mechanism that penalizes the claimant for travelling throughout the CTA to exercise their right to healthcare contradicts the bilateral commitments established by both sovereign states.⁸

Clinical Diagnostics and the Necessity of Trauma-Informed Psychiatry

The primary driver of the claimant's extraterritorial travel is the clinical necessity of trauma-informed psychiatric care, a specialized modality that the claimant reports is structurally deficient within the Irish public health apparatus. The claimant was formally diagnosed with

Anxiety Disorder and Post-Traumatic Stress Disorder (PTSD) in September 2017 by a university psychiatrist, a diagnosis that formed the foundational basis for the claimant's receipt of the Irish Disability Allowance.³

However, the clinical reality of the claimant's pathology is more accurately defined as Complex Post-Traumatic Stress Disorder (CPTSD). Rooted in prolonged, repeated trauma—specifically severe childhood adversity and severe domestic violence inflicted by family members—CPTSD manifests through profound neurobiological dysregulation. The claimant reports symptoms aligning with established CPTSD literature, including severe emotional flashbacks, an overactive "tyrannical self-critic," and highly heightened physiological fight/flight/freeze/fawn responses.³

Trauma-informed care (TIC) represents a systemic clinical shift from treating isolated behavioural symptoms to addressing the underlying neurobiological impact of adversity, violence, and trauma.⁹ While TIC principles are recognized in global health policies, implementation within public health systems like the Irish HSE remains highly fragmented and often inaccessible for patients requiring highly individualized pathways.¹¹ Consequently, the claimant determined that navigating the public healthcare systems of Ireland would be entirely "limiting" and would preclude access to a clinician specifically specialized in complex trauma.³ To secure appropriate, life-sustaining care, the claimant relocated temporarily to London in 2025 to engage the UK private healthcare sector.³ Communications with the London Psychiatry Clinic reveal a highly targeted search for specialized intervention.

Table 1: Psychiatric Consultation and Clinic Engagement Timeline

Date	Entity	Communication/Action Summary	Reference
September 2017	Irish University Health Unit	Original diagnosis of Anxiety Disorder and PTSD established.	³
25 September 2025	London Psychiatry Clinic	Initial enquiry response from clinic coordinator Charlotte Winnock.	³
22 October 2025	London Psychiatry Clinic	Claimant provides detailed CPTSD history; explicitly requests consultation with Dr. Deirdre MacManus due to her expertise with trauma and incarcerated populations.	³
24 October 2025	London Psychiatry Clinic	Clinic initially states Dr. MacManus is	³

		unavailable until early 2026. Later clarifies that Dr. MacManus lacks capacity for "complex presentations."	
27 October 2025	London Psychiatry Clinic	Clinic recommends Dr. ³ Tamara Ventura Wurman for CPTSD. Initial 90-minute consultation fee quoted at £870; subsequent follow-up care quoted at £580–£595 per hour.	

The clinical correspondence underscores the severity and complexity of the claimant's condition. The London Psychiatry Clinic formally advised that given the claimant's presentation, a brief intervention of one or two appointments would be clinically unsafe; rather, "ongoing follow-up care would be essential" to ensure the patient's safety and wellbeing.³

The implications of this clinical trajectory are profound. The claimant's travel to the UK is unequivocally driven by medical necessity. As a British citizen, the claimant is exercising their fundamental right to access the UK's advanced private psychiatric market. The financial burden of this care is substantial, requiring significant capital output for initial assessments (£870) and ongoing therapy (£580–£595 per hour).³ Any attempt by the Irish Department of Social Protection to terminate the claimant's Disability Allowance due to the very travel required to access this care would be counter-therapeutic, economically devastating, and a violation of the claimant's CTA protections.

Pharmacological Discrepancies: Medical Cannabis and Vaporisation Technologies

The claimant's treatment protocol features the use of medical cannabis, prescribed to mitigate the severe hyperarousal, persistent anxiety, and insomnia secondary to CPTSD.³ Similar to the psychiatric care paradigm, the regulatory landscape surrounding medical cannabis creates an absolute geographical imperative for the claimant to access the UK healthcare system, as domestic Irish options are statutorily foreclosed.

In 2019, the Republic of Ireland introduced the Medical Cannabis Access Programme (MCAP) on a pilot basis. However, the clinical parameters of the MCAP are profoundly restrictive. It permits specialist consultants to prescribe cannabis-based products solely for three highly specific indications: spasticity associated with multiple sclerosis, intractable nausea and vomiting associated with chemotherapy, and severe, treatment-resistant epilepsy.⁵ Psychiatric conditions, including PTSD, CPTSD, and treatment-resistant anxiety, are entirely excluded from the Irish framework.¹³ Consequently, patients suffering from severe psychiatric trauma in Ireland

cannot legally access medical cannabis through domestic public health channels, leaving them vulnerable to criminalization or forced reliance on unregulated illicit markets.¹⁴

In stark contrast, the United Kingdom adopted an evidence-based policy shift in November 2018, rescheduling cannabis-based products for medicinal use (CBPMs) to Schedule 2 of the Misuse of Drugs Regulations 2001, granting specialist doctors the authority to prescribe them for a wide array of conditions.¹⁴ While access via the public NHS remains exceptionally limited due to a lack of licensed products and funding constraints, a robust private clinic ecosystem has emerged across the UK.¹⁶ These Care Quality Commission (CQC) registered clinics routinely prescribe medical cannabis for psychiatric conditions, including PTSD, generalized anxiety disorder, and sleep disorders linked to trauma.¹² Real-world observational studies, such as the UK Medical Cannabis Registry and Project Twenty21, have documented statistically significant improvements in PTSD-specific symptoms (IES-R scores), anxiety (GAD-7), and sleep quality (SQS) among patients utilizing CBPMs under clinical supervision.¹⁷

The documentary evidence confirms the claimant's active and continuous engagement with the UK medical cannabis sector.

Table 2: Medical Cannabis Clinic Engagement

Date	Clinic	Action / Status	Reference
16 March 2024	Curaleaf Clinic	Update on treatment enquiry; clinic awaiting Summary of Care Record / Emergency Care Record to proceed with medical assessment.	³
13 October 2025	Alternaleaf Clinic	Confirmation of patient registration; clinic confirms patient is eligible for medical cannabis treatment based on shared information and invites booking for initial assessment.	³

Furthermore, the claimant has procured specialized medical vaporizers and high-grade cannabidiol (CBD) and cannabigerol (CBG) botanicals from UK and European dispensaries. The use of advanced vaporizers—specifically the Storz & Bickel Mighty+ and Venty (which hold medical device certifications) and thermal extraction devices like the DynaVap—aligns with clinical guidelines that mandate the inhalation of heated vapor rather than the combustion of cannabis flower. Vaporisation minimizes respiratory toxicity while ensuring the rapid onset of anxiolytic effects necessary to halt acute emotional flashbacks in CPTSD patients.

Table 3: Botanical Procurement and Vaporiser Device Acquisitions

Date of Order	Vendor	Delivery Location	Items Procured	Reference
11 November 2020	Vapefiend	Newtownabbey, BT36 4PE	Dynavap AzuriuM "M" 2020, SnapStash, Grinder	³
21 November 2022	DynaVap EU	Galway, H91 CYH5	The "M" AzuriuM, DynaCoil	³
26 December 2023	Little Collins CBD	Galway, H91 CYH5	Little "Cali" GELATO, Cookie Dough HASH	³
28 December 2023	MagicVaporizers	Galway, H91 CYH5	Mighty+ (Plus) Vaporizer, Capsule Caddy, Drip Pad	³
12 July 2024	Iarnród Éireann	Galway Ceannt	Travel record indicating transit corresponding to botanical needs.	³
19 August 2024	HempHash	Belfast, BT12 7AS	Mystery CBD Flower (28g), CBD+CBG Hemp Hash, Mixed Shake	³
19 August 2024	Herbalize Store UK	Belfast, BT12 7AS	DynaVap - The B	³
10 February 2025	Little Collins CBD	Galway, H91 KN7F	Fitzroy Street Harlequin, Planetary Bent CBD, Full Spectrum CBN/CBD Oil, Full Spectrum CBG/CBD Oil	³
19 February 2025	Little Collins CBD	Galway, H91 KN7F	Cali HAZE, Korova ORANGE, Hemp CBD SMALLS	³
28 February 2025	Mama Kana	Galway, H91 KN7F	Huile de Chanvre BIO CBN, Huile de Chanvre BIO CBG, CBD flowers	³

18 May 2025	Little Collins CBD	Galway, H91 KN7F	Mission Beach Mango, Little "Cali" Gelato CBD Bud	³
17 August 2025	MagicVaporizers	London, NW1 9HZ	DynaVap M7, Ispire - The Wand (Induction Heater)	³
08 September 2025	Vapefiend	Belfast, BT12 7AS	Dynavap M7/M7XL Vaporizer	³
22 January 2026	HempHash	London, N7 0LU	Cannatonic CBD Hemp Flower	³
25 March 2026	Vapefiend	London, N7 0LU	VENTY Vaporizer, Magazine with Dosing Capsules	³

The structural divergence between the Irish MCAP and the UK private clinical framework creates an absolute dependency on cross-border travel. The claimant cannot receive holistic, evidence-based trauma care, including legally prescribed medicinal cannabis for PTSD, in the Republic of Ireland. The CTA guarantees the claimant's right to cross the border into Northern Ireland (Belfast) or Great Britain (London) to consult with private cannabis clinicians, obtain prescriptions for CPTSD, and procure the necessary botanical products and hardware.³

Advanced Therapeutics: Neuropeptide and Nootropic Procurement

In conjunction with psychiatric intervention and cannabinoid therapies, the claimant's treatment protocol involves the sustained administration of synthetic neuropeptides. These compounds are utilized to address neuroinflammation, expedite cellular repair mechanisms, and manage the profound neurological dysregulation stemming from psychological trauma and brain injury. The procurement of these substances exposes another stark regulatory asymmetry between the Republic of Ireland and the United Kingdom, necessitating the claimant's reliance on UK delivery addresses and the CTA framework.

The claimant's order history demonstrates the sustained acquisition of several powerful therapeutic peptides. Chief among these is BPC-157 (Body Protection Compound 157), a synthetic 15-amino-acid peptide derived from human gastric juice, widely cited in pharmacological literature for its capacity to accelerate soft tissue repair, modulate the central nervous system, and exert neuroprotective effects.²⁰ Additionally, the claimant utilizes intranasal peptides such as Selank and Semax. These formulations are heavily utilized for their potent anxiolytic, neurorestorative, and cognitive-enhancing effects, offering a highly efficient delivery mechanism that bypasses the blood-brain barrier with low inherent toxicity.²¹ Furthermore, the procurement includes Cerebrolysin and Cortexin, complex peptide mixtures featuring neurotrophic factors utilized to support neurogenesis and cognitive recovery.³

Within the Republic of Ireland, the Health Products Regulatory Authority (HPRA) maintains a

highly restrictive, enforcement-heavy posture regarding unauthorized medicinal products. Peptides such as BPC-157, Selank, and Semax lack formal marketing authorization from the European Medicines Agency (EMA).²² Consequently, the HPRA classifies the importation of these substances by private individuals as an illegal act.⁴ In 2025 alone, the HPRA detained over 763,000 units of falsified, unauthorized, and illegal medicines at the border, actively seizing shipments and occasionally supporting prosecutions.⁴ Importing these essential therapeutics directly to the claimant's residence in Galway is therefore unviable, carrying an unacceptable risk of customs seizure, financial loss, and legal penalty.²²

Conversely, the United Kingdom operates under a more nuanced regulatory interpretation. While the Medicines and Healthcare products Regulatory Agency (MHRA) strictly prohibits vendors from marketing peptides with explicit medicinal claims for human use, the substances themselves are not controlled narcotics under the Misuse of Drugs Act 1971.²³ Consequently, a legal "grey area" exists wherein UK-based vendors can legally sell these neuropeptides domestically, provided they are labeled strictly as "research chemicals" not for human consumption.²⁴

To legally navigate this jurisdictional arbitrage, the claimant utilizes their status as a British citizen to order these therapeutics to addresses within the UK. The documented purchase history confirms this complex logistical workaround:

Table 4: Neuropeptide and Neurogenic Therapeutic Procurement

Date of Order	Vendor	UK Delivery Location	Therapeutics Procured	Reference
12 August 2024	UK Peptides	Belfast, BT12 7AS	BPC-157, Bacteriostatic Mixing Water	³
23 December 2024	UK Peptides	Belfast, BT9 5AD	BPC-157, Selank, Semax, DSIP, CJC-1295, Ipamorelin, Mixing Water	³
24 December 2024	OTC Online Store	Belfast, BT9 5AD	Cerebrolysin, Cortexin	³
17 July 2025	OTC Online Store	Belfast, BT9 5AD	Cerebrolysin, Cortexin	³
24 September 2025	UK Peptides	London, NW1 9HZ	BPC-157, Selank, Semax, Mixing Water	³
30 October 2025	UK Peptides	London, N7 0LU	BPC-157, Selank, Semax, DSIP, Thymosin Beta 4, MOTS-C,	³

			CJC-1295, Ipamorelin	
02 February 2026	OTC Online Store	London, N7 0LU	Cerebrolysin, Cortexin	³
11 February 2026	UK Peptides	London, N7 0LU	BPC-157, Selank, Semax	³
25 February 2026	UK Peptides	London, N7 0LU	BPC-157, Selank, Semax, Mixing Water	³

This chronological dataset reveals a distinct shift in geographical delivery patterns, moving from utilizing Belfast residential and out-of-home locker networks (OOHPod) in 2024 and mid-2025, to direct residential delivery in London from late 2025 into 2026.[3, 3] This directly correlates with the claimant's relocation to London to seek psychiatric care from the London Psychiatry Clinic.³ The underlying implication is irrefutable: the claimant's travel to Belfast and London is a mandatory logistical requirement to secure neuro-regenerative treatments blockaded by the Irish State. The travel is intrinsically linked to the proactive management of their disability.

Logistical Topography: Evidence of Extensive Cross-Border Transit

The integration of private psychiatric care, medical cannabis consultations, and neuropeptide procurement necessitates a highly structured, frequent, and demanding travel itinerary. The claimant suffers from CPTSD linked to domestic violence and physical assaults, which has resulted in a documented fear of flying, thereby eliminating aviation as a viable mode of transport.³ As a result, the claimant engages in a "nomadic lifestyle," relying entirely on surface transit—a complex network of rail infrastructure and maritime ferries operating across the CTA.³ The travel topography involves frequent navigation between Galway (the claimant's place of origin, familial base, and location for academic commitments at Ollscoil na Gaillimhe), Dublin, Belfast, Holyhead, Liverpool, and London.³ The documented evidence of train travel operated by Iarnród Éireann (Irish Rail) confirms an extraordinarily extensive history of movement, specifically concentrated on two primary corridors: Dublin Heuston to Galway Ceannt, and Dublin Connolly to Belfast.

Visual evidence of travel tickets and receipts further substantiates this complex Sail & Rail topography, explicitly detailing the multi-carrier coordination required (Avanti West Coast, Transport for Wales, Stena Line, and Irish Ferries) to maintain movement between Dublin, Holyhead, and London.

Table 5: Exhaustive UK Ferry and Rail Logistics (Visual & Document Evidence)

Date	Entity / Route	Document / Details	Reference
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10 August 2025	Stena Line	E-ticket & Onboard Receipt: Dublin (T2) to Holyhead (T5) (02:15 departure; onboard purchase at 02:39)	³ , Visual Evidence
24 August 2025	SailRail / Irish Ferries	Master Ticket: London Terminals to Dublin Ferryport via Holyhead (£60.10)	Visual Evidence
24 August 2025	Avanti West Coast	Seat Reservation: London Euston to Crewe (20:13)	Visual Evidence
24 August 2025	Transport for Wales	Seat Reservation: Crewe to Holyhead (22:29)	Visual Evidence
25 August 2025	Transport for Wales / Irish Ferries	Seat Reservation / Ferry: Holyhead to Dublin Ferryport (02:40)	Visual Evidence
27 August 2025	Transport for Wales / Stena Line	Seat Reservation / Ferry: Dublin Port Stena to Holyhead (02:15)	Visual Evidence
27 August 2025	Avanti West Coast	Seat Reservation: Holyhead to London Euston (06:48)	Visual Evidence
10 September 2025	Stena Line	Boarding Card: Holyhead (T5) to Dublin (T2) (02:00)	Visual Evidence
11 September 2025	Iarnród Éireann / Stena Line	Sail & Rail Ticket (Issued Dublin Port, €64) & Boarding Card: Dublin (T2) to Holyhead (T5) (22:05)	Visual Evidence
30 March 2026	Avanti West Coast	Seat Reservation: London Euston to Holyhead (08:00)	Visual Evidence
30 March 2026	Transport for Wales / Irish Ferries	Seat Reservation / Ferry: Holyhead to Dublin Ferryport (13:15)	Visual Evidence
30 March 2026	Irish Ferries	Boarding Card	Visual Evidence

		(Ulysses) & Baggage Reclaim Tags (Dublin No. 001182, 001183): 1 Adult (13:15 departure)	
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Table 6: Rail Transit History (Dublin - Belfast Corridor)

Date of Travel	Route	Departure	Arrival	Reference
16 March 2024	Dublin Connolly to Belfast	11:20	13:35	3
18 March 2024	Dublin Connolly to Belfast	19:00	21:05	3
20 March 2024	Belfast to Dublin Connolly	06:50	09:02	3
23 March 2024	Dublin Connolly to Belfast	19:00	21:05	3
24 March 2024	Belfast to Dublin Connolly	13:05	15:15	3
24 March 2024	Dublin Connolly to Belfast	19:00	21:08	3
25 March 2024	Belfast to Dublin Connolly	12:35	14:42	3
29 March 2024	Dublin Connolly to Belfast	20:50	23:00	3
01 April 2024	Belfast to Dublin Connolly	12:20	14:45	3
05 April 2024	Dublin Connolly to Belfast	20:50	23:00	3
09 April 2024	Belfast to Dublin Connolly	14:05	16:20	3
12 April 2024	Dublin Connolly to Belfast	19:00	21:05	3
15 April 2024	Belfast to Dublin Connolly	16:05	18:17	3
16 April 2024	Belfast to Dublin Connolly	14:05	16:20	3
19 April 2024	Dublin Connolly to Belfast	19:00	21:05	3
29 April 2024	Dublin Connolly to Belfast	16:50	19:05	3
02 May 2024	Dublin Connolly to Belfast	11:20	13:35	3

02 May 2024	Belfast to Dublin Connolly	18:05	20:15	3
03 May 2024	Dublin Connolly to Belfast	11:20	13:35	3
03 May 2024	Belfast to Dublin Connolly	14:05	16:20	3
05 May 2024	Belfast to Dublin Connolly	11:05	13:15	3
12 July 2024	Dublin Connolly to Belfast	07:35	09:48	3
12 July 2024	Belfast to Dublin Connolly	13:50	16:20	3
13 July 2024	Dublin Connolly to Belfast	07:35	09:48	3
13 July 2024	Belfast to Dublin Connolly	13:50	16:20	3
17 July 2024	Dublin Connolly to Belfast	09:30	11:45	3
29 July 2024	Belfast to Dublin Connolly	13:50	16:20	3
01 August 2024	Dublin Connolly to Belfast	10:50	12:58	3
09 August 2024	Dublin Connolly to Belfast	07:35	09:48	3
09 August 2024	Belfast to Dublin Connolly	12:20	14:40	3
12 August 2024	Dublin Connolly to Belfast	09:30	11:45	3
13 August 2024	Dublin Connolly to Belfast	09:30	11:45	3
13 August 2024	Dublin Connolly to Belfast	16:50	19:05	3
16 August 2024	Belfast to Dublin Connolly	12:20	14:40	3
07 January 2025	Dublin Connolly to Belfast	05:50	07:58	3
07 January 2025	Belfast to Dublin Connolly	13:00	15:13	3
17 July 2025	Dublin Connolly to Belfast	13:50	15:58	3
09 August 2025	Dublin Connolly to Belfast	05:50	07:58	3

09 August 2025	Belfast to Dublin Connolly	12:00	14:15	3
09 August 2025	Dublin Connolly to Belfast	15:50	17:58	3
09 August 2025	Belfast to Dublin Connolly	19:00	21:15	3
10 September 2025	Dublin Connolly to Belfast	20:50	22:58	3
11 September 2025	Dublin Connolly to Belfast	05:50	07:58	3
11 September 2025	Belfast to Dublin Connolly	09:00	11:13	3
04 February 2026	Dublin Connolly to Belfast	13:50	15:58	3

Table 7: Rail Transit History (Dublin - Galway Corridor)

Date of Travel	Route	Departure	Arrival	Reference
02 March 2024	Galway Ceannt to Dublin Heuston	07:30	09:49	3
03 March 2024	Dublin Heuston to Galway Ceannt	14:40	17:07	3
04 March 2024	Dublin Heuston to Galway Ceannt	19:35	22:00	3
10 March 2024	Dublin Heuston to Galway Ceannt	14:40	17:07	3
24 March 2024	Dublin Heuston to Galway Ceannt	16:35	19:10	3
25 March 2024	Dublin Heuston to Galway Ceannt	15:35	18:00	3
01 April 2024	Dublin Heuston to Galway Ceannt	16:30	18:51	3
05 April 2024	Galway Ceannt to Dublin Heuston	17:20	19:58	3
09 April 2024	Dublin Heuston to Galway Ceannt	17:30	20:04	3
12 April 2024	Galway Ceannt to Dublin Heuston	15:05	17:40	3
12 April 2024	Galway Ceannt to Dublin Heuston	17:20	19:58	3
16 April 2024	Dublin Heuston to Galway Ceannt	17:30	20:04	3

03 May 2024	Dublin Heuston to Galway Ceannt	16:30	18:51	³
12 July 2024	Dublin Heuston to Galway Ceannt	17:30	20:04	³
07 January 2025	Dublin Heuston to Galway Ceannt	16:30	18:54	³
11 April 2025	Dublin Heuston to Galway Ceannt	16:30	18:54	³
11 April 2025	Dublin Heuston to Galway Ceannt	18:30	20:56	³
25 August 2025	Dublin Heuston to Galway Ceannt	17:30	20:07	³
11 September 2025	Dublin Heuston to Galway Ceannt (Day Return)	-	-	Visual Evidence
30 March 2026	Dublin Heuston to Galway Ceannt	18:30	20:59	³
30 March 2026	Dublin Heuston to Galway Ceannt	19:35	22:14	³
01 April 2026	Dublin Heuston to Galway Ceannt	17:30	20:10	³

The sheer density of this travel schedule highlights a critical third-order insight: the claimant's movement is not indicative of leisure or prolonged holiday absences. For instance, the itinerary on 11 September 2025 demonstrates a turnaround time in Belfast of merely one hour and fifteen minutes before returning to the Republic of Ireland.³ Similarly, the July 12 and 13, 2024 itineraries show back-to-back day trips to Belfast lasting mere hours. Such rapid cross-border transit is characteristic of precise logistical operations designed exclusively to retrieve medical packages, attend brief clinical consultations, or access dispensaries located within the Northern Irish jurisdiction of the UK.

Furthermore, the claimant explicitly records travel via the "sail and rail" ferry networks connecting Dublin and Belfast to Holyhead and Liverpool, culminating in rail transit to London.³ The claimant maintained a presence in Islington, London, during late 2025 and early 2026 to engage directly with the London Psychiatry Clinic and to facilitate the receipt of specialized medications to local addresses.[3, 3] The claimant is simultaneously integrated into UK civic life, evidenced by political memberships in the Alliance Party in Belfast, the Liberal Democrats in Liverpool/London, alongside Fine Gael in Galway.³ The necessity of maintaining these multiple touchpoints—one in Ireland for familial ties and educational commitments, and several across the UK for medical access and civic engagement—is a direct consequence of the regulatory and clinical asymmetries between the two states.

The claimant's reliance on Stena Line, Irish Ferries, and Avanti West Coast sail-and-rail routes from Holyhead to London is a mandatory accommodation for a patient suffering from CPTSD-induced aviation phobia.³ Consequently, travel to London from Galway takes

significantly longer than a standard flight, demanding longer absences from the State simply to physically access the UK clinical sites.

The Human Cost: Benefit Cessation, Starvation, and Emergency Universal Credit

The rigid application of social welfare regulations has had a catastrophic impact on the claimant's wellbeing and fundamentally undermined the purpose of the Disability Allowance. The claimant proactively telephoned the Disability Allowance office multiple times in July 2025, explicitly outlining the severe domestic violence they were fleeing and the absolute necessity of changing their address to safely escape. During these communications, the claimant arranged to change their registered postal address to an OOHPod locker in Dundalk to facilitate the secure reissue of their Public Services Card. Instead of providing trauma-informed administrative support or recognizing these exceptional circumstances, the Department failed to protect the claimant. It was only later, after the office was illegally contacted by an outside party in March 2026, that they initiated an abrupt and illegal cessation of the Disability Allowance on 14 March 2026, executing this termination without any prior warning or contact with the claimant.

This uncommunicated termination plunged the claimant into absolute destitution, resulting in severe starvation, profound humiliation, and an immediate threat to their physical survival. Documentary evidence from March 2026 reveals the severe consequences of this economic blockade, showing the claimant's reliance on emergency food support in London, including desperate communications with local charities such as the Hargrave Hall Community Association and the Copenhagen Street Foodbank to secure a free hot meal.³

To survive this state-induced starvation, the claimant was forced to undergo the deeply retraumatizing process of applying for an emergency Universal Credit claim in the UK on 19 March 2026.³ This retraumatisation was severely compounded by the Irish HSE's refusal to release the claimant's valid medical notes, erecting further barriers to receiving aid.³

The Universal Credit journal confirms that the claimant received only one payment before voluntarily ceasing the claim on 14 May 2026.³ Visual evidence of the official closure request details the compounding trauma the claimant endured during this period. The claimant recorded that shortly after receiving an advance payment, they were subjected to further verbal abuse in their shared flat and faced a retaliatory eviction by their landlord. Unable to secure a solicitor to defend against the eviction, with their CPTSD worsening, and facing threats to their life by their brother, the claimant was forced to return to Ireland in early April 2026 to act as a carer for their distressed grandmother. The claimant emphasizes that while they retain an inherent legal right to claim Universal Credit as a British citizen traversing the CTA, Universal Credit is a broader, means-tested benefit and is distinctly separate from the Personal Independence Payment (PIP), which serves as the true UK equivalent to the Irish Disability Allowance. The forced reliance on an emergency Universal Credit claim due to the sudden, illegal termination of the Irish Disability Allowance—all while fleeing domestic violence—stands as a stark indictment of the Department's failure to protect a highly vulnerable dual citizen actively seeking to manage their

CPTSD.

Statutory Analysis: Social Welfare Legislation and CTA Protections

The critical friction point in this case resides in the domestic social welfare legislation of the Republic of Ireland. The claimant is in receipt of the Disability Allowance (DA), a means-tested social assistance payment designed to support individuals whose disability substantially restricts their capacity to work.²⁵

Under Section 249(1) of the Social Welfare Consolidation Act 2005, a person is generally disqualified from receiving benefit payments for any period during which they are absent from the State.²⁷ This physical presence requirement is strictly enforced, and unauthorized absences routinely lead to the suspension or termination of payments.²⁸

However, the statutory rigidity of Section 249 is tempered by explicit legislative and administrative exceptions. Section 249(3) and 249(4) empower the Minister to draft regulations that suspend or modify the disqualification in prescribed cases.²⁷ Crucially, the Department of Social Protection's Operational Guidelines acknowledge a long-standing administrative exception allowing for the payment of Disability Allowance while a claimant is outside the State for the specific purpose of receiving medical treatment.²⁹

Historically, the Department has stipulated that this exception is granted if the medical treatment is not available in the State, if the person is referred by the HSE, and if there is confirmation of treatment.³⁰ This bureaucratic interpretation frequently aligns with the parameters of the HSE Treatment Abroad Scheme (TAS) or the Cross-Border Healthcare Directive.³¹ However, the TAS is explicitly restricted to public healthcare patients; referrals from private consultants or applications to fund private healthcare abroad are strictly ineligible under the HSE TAS.³¹ Because the claimant is accessing *private* trauma-informed psychiatry in London, *private* neuropeptide acquisition, and *private* medical cannabis prescriptions in the UK, they sit entirely outside the traditional HSE-approved Treatment Abroad Scheme architecture.³¹

This is precisely where the claimant's status as a British citizen under the Common Travel Area supersedes standard domestic administrative guidance. The 2019 CTA Memorandum of Understanding guarantees that British citizens residing in Ireland have the absolute right to access health services in the UK on the exact same basis as citizens of that state.³³ The Withdrawal Agreement and subsequent UK-EU treaties further protect the reciprocal nature of social security and healthcare access.³⁴

If the Irish Department of Social Protection were to rigidly apply the Section 249 disqualification to the claimant, it would effectively weaponize the Disability Allowance to construct an economic blockade against the claimant's legally protected CTA rights. The claimant is travelling to the UK to receive medically necessary therapies (trauma-informed CPTSD psychiatry, medical cannabis, neuro-regenerative peptides) that the Irish State has structurally failed to provide and legally restricted. The absence from the State is therefore entirely for the purpose of medical treatment.

To deny the administrative exception for medical travel simply because the treatment is "private"

and located in the UK would represent a highly discriminatory application of the Social Welfare Consolidation Act 2005 against a dual national. The claimant's UK citizenship grants an inherent legal right to interface with the UK's private health sector and its associated, more permissive regulatory frameworks (e.g., the MHRA's tolerance of peptide research chemicals and the Schedule 2 prescribing of CBPMs). The Irish State cannot legitimately demand unbroken physical presence as a condition of disability support if that presence forces the claimant to endure sub-standard psychiatric care, blocks access to international pharmacological relief, and fundamentally infringes upon their rights as a British citizen traversing the Common Travel Area.

Concluding Synthesis

The evidentiary record confirms unequivocally that the claimant, a dual Irish-British citizen, engages in extensive cross-border travel utilizing the rail and ferry networks spanning Galway, Dublin, Belfast, Holyhead, and London. This intense travel topography is not recreational; it is a highly coordinated logistical necessity driven by a severe diagnosis of Complex Post-Traumatic Stress Disorder and the absolute requirement to access advanced therapeutics.

The Republic of Ireland's healthcare infrastructure presents insurmountable barriers to the claimant's recovery. The HSE's structural deficit in trauma-informed psychiatric pathways necessitates the claimant's engagement with leading private experts such as those at the London Psychiatry Clinic. Furthermore, the HPRA's aggressive prohibition of neuro-regenerative peptides and the extreme exclusivity of the Irish Medical Cannabis Access Programme structurally forces the claimant to utilize UK delivery addresses and UK-based private clinicians. As a British citizen, the claimant possesses an unassailable right under the Common Travel Area to access the healthcare and commercial frameworks of the United Kingdom without hindrance. The travel undertaken to Belfast and London is therefore a legally protected exercise of jurisdictional rights for the explicit purpose of medical treatment.

Consequently, the Department of Social Protection must interpret the claimant's travel history through the lens of CTA obligations and the established administrative medical exceptions to Section 249 of the Social Welfare Consolidation Act 2005. The claimant's absences from the Republic of Ireland are medically essential and protected by international treaty. Any suspension, reduction, or revocation of the Disability Allowance based on these necessary medical absences would constitute an unjust impediment to the claimant's fundamental right to health, and a severe breach of the reciprocal privileges afforded to British citizens residing in Ireland. The continuous provision of the Disability Allowance must be maintained to ensure the claimant possesses the financial capability required to secure their ongoing medical stability across the Common Travel Area.

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